

I: Now we'll start with the actual questions. The first one is about familiarity with AI. How familiar are you with AI technologies in general?

P: I think that also depends on how you define familiarity, but I know about ChatGPT and I know how to look things up. I've used it a little bit, but mostly for fun. Like with friends, someone is having a baby and we ask ChatGPT, given the names of the parents and their other kids, what names would you recommend, or writing vows, stuff like that. Sometimes I've used it to research something, but I don't use it a lot. I know a lot of people who use it a lot more than I do. I'm also a little bit hesitant to use it because I know it takes quite a lot of energy and often leads to mistakes. And I sometimes come across it when I teach for the [POST-ACADEMIC PROGRAM]. It has happened that I've gotten a report from a student and I think, this is not you, this is AI. I hate that because I ask them to write reflections about their thinking, and if I get something made by AI then it's no use. So sometimes it annoys me a little bit.

I: In your organization, have you introduced AI at all?

P: No, not really. I work by myself, but there are two companies I work with. One of them I work with because if you have your own practice it's pretty hard to give people mental health care that's paid for by insurance, as there are so many things you need to do before insurance will cover your sessions. Through this company I tick all the boxes and can do sessions paid for by insurance. I know they have a pilot going on where AI listens to your sessions and then makes a summary, and I have not applied for that.

I: What company is this?

P: It's called [COMPANY]. It's a company that specifically works with psychologists who work by themselves, who have experience and are able to work with people in the first line of basic mental health care, sessions paid for by the government, but who by themselves are not recognized by mental health care as professionals. [COMPANY] handles the whole administrative side of things. They also saw that there are so many waiting lists and so many people waiting for treatment, and there's also a big group of psychologists who are able to treat these people but aren't allowed because of certain boxes they don't tick. So they built an organization to connect these two groups, which I think is really good.

I: To finish this question off, can you give a score of 1 to 10 on how familiar you are with AI?

P: I'm guessing a two or three.

I: Then for the next question, this is about diagnostics and assessment use. What are your thoughts on AI being used in diagnostic or assessment processes?

P: How would you use them?

I: I also don't want to say too much because I don't want to steer the conversation, but in psychology you want to diagnose people, and you have tests for that or you have an intake, and

there might be some opportunities for AI to help. For example, you mentioned that AI could summarize things, but there might also be other things it could do, like saying this person has this disorder. There could be a wide range of applications. What are your thoughts, and maybe some things you think it should or shouldn't be doing?

P: One thing you said is that we want to diagnose people. Well, I don't necessarily want to diagnose people. Sometimes it can be useful, but we need to do it because the insurance companies want us to put a label on it so they can decide to pay or not. That's mostly why we do the diagnosis. Using AI for that, I think there's a part of it you could use, maybe to form your opinion a little further. But forming a diagnosis is complex, because you're looking at symptoms that somebody is experiencing, and a lot of symptoms can point to so many different diagnoses. For example, if I work with an athlete who is really strict about sleeping times and eating habits and is very rigorous about that, there might be a chance AI says this is autism, since there are some symptoms in there. But those are also things you sometimes need in elite sports to perform well. In the sports context it's not necessarily a symptom of autism, while in another context it could be. I'm not sure whether AI can truly understand these different contexts, because it feels like it mostly looks at symptoms and tries to find where those symptoms appear together in an overview, and then puts a diagnosis on it. So I think you could maybe use it to get some ideas, put a list of symptoms into AI and see what it comes up with diagnostically. That might be quicker than getting your DSM and looking at all the different diagnoses yourself, so maybe it can save a little bit of time. But there are also things that you sense or see, body language, non-verbal cues, things that are hard to put into words but that you do register as a psychologist, and I think it's hard to make AI understand the relevance of that. So to make an actual diagnosis I wouldn't trust AI, but to brainstorm and get some ideas, I might use it.

I: For diagnosing, do you actually use the DSM for every case?

P: Not the book itself. There are some diagnoses I've used so much that it's pretty clear, like anxiety disorders or depressive disorders, I know the most important symptoms and sometimes it's just really clear. Post-traumatic stress disorder as well, I see that quite a lot and it's not too hard to work with. But sometimes when it's symptoms that are a bit more unusual or I don't come across them often, or there's a combination where I'm not sure what is going on, then I might use the DSM or the internet. Actually I think I use the internet more than the book itself, but I try to look for reliable websites that have the same criteria.

I: Is there a specific website you look at, or is it just Googling symptoms?

P: I could do both. There's a Dutch website with the guidelines in mental health care, all types of different disorders according to DSM-5, and I try to use that. You can Google, but you also get a lot of rubbish, websites where people are trying to diagnose themselves, so that's not really useful. I might use it and then filter for which websites are reliable. Nowadays you also get that little AI prompt at the top of search results, and I use that sometimes, but for making diagnoses I'm pretty careful with that. In my spare time I might use it more for less important things, but for making diagnoses, yeah, I'm careful.

I: Then for the next question, this is about monitoring and progress tracking. How do you view the role of AI in tracking patient progress over time?

P: I don't think it has a role in my work. I think the only way to do that would be to let AI listen to all of your sessions and then ask it whether it sees a difference, or make a graph, you can probably ask AI to do stuff like that. But I don't trust it enough to let it listen to my sessions. And I think if I would ask AI to make a summary of my sessions, it would not be a summary I would find useful, because it would probably just be a summary of what we talked about. What I write down as a summary is more interpretations, more points of view, a little bit about how the client is doing. If I do an exercise with a client, I don't need a whole description of the exercise because I know how I did it. What I would write down is how the client responded, what the key moments or most emotional moments were, or what the takeaway message was for the client. I think it's hard to teach AI to get the things from the session that I need, so I wouldn't really see how I could use a summary from AI. Maybe you would need to write a very specific prompt for that. But I also write along a little bit during sessions and it helps me stay focused, so the note-taking itself has a purpose during the session as well.

I: You mentioned that you don't trust AI for these kinds of things. Is there a specific part of that trust that you think is a limiting factor?

P: I promise my clients confidentiality, and if there's something listening along and I'm not completely sure where that data goes or if someone can buy it, then I can't promise that.

I: So it's mostly focused on the privacy part. Do you actually do any monitoring with your clients, or is that not really used that much?

P: Verbally, I do that a lot of course. With questionnaires, there's often one I do when we start and when we finish. Because you hope that at the end the client verbally tells you they are doing better, but it's also really helpful to have a questionnaire that solidifies that and shows you the person is actually doing better. Often people at the end have sort of forgotten how they were feeling at the beginning. So sometimes when you look at those two questionnaires together, the one from the beginning and what's happening now, they're like, oh yeah, that's how I felt. Even though it's sometimes four months in the past and they don't have a very clear recollection of that. I think that's a helpful tool, but I don't use a questionnaire every session.

I: And not a questionnaire in the middle? With the previous psychologists I've talked to they do a start, middle, and end, but you mainly focus on start and end?

P: For this questionnaire I do, but the one I'm talking about measures symptoms that people experience, so that's looking at how many symptoms of a disorder are present, depressive symptoms, anxiety symptoms, that kind of thing. Verbally, what I often do is somewhere halfway through we have an evaluation where we look at how you think everything is going, what you think of the sessions, what you think of the method we're using, how far you think you are in your process. I sometimes ask them to give me a percentage on where they are. But it's not at a

very set moment, often it occurs naturally in the treatment, or I'm curious at a certain point, or at a point where I'm not completely sure what the next step is, and then I do an evaluation as well.

I: Then for the next question, this is about administrative and practical support. How do you feel about AI generating drafts or reports or assisting with scheduling and reminders?

P: I think you could use it for that, but you also need to look at the confidentiality issues. Maybe if you would do it without any names, then you could ask AI to do that. But often when I have to write a report there's already a format for it, and there are certain sentences I use again in different contexts, so it doesn't take me that much time. I don't write that many reports that I need a solution for this. So I'm not really looking for that, but I think you could use AI for reports if you take anonymity and confidentiality seriously. I would always read it over, and I'm curious whether I would be happy with it, but I would just have to try it to find out.

I: With scheduling in mind, you work with this company, do they plan your schedule or do you do it yourself?

P: Nobody touches my schedule, I like to do that myself. I know some colleagues have a system where clients can see their calendar and put an appointment in themselves, and that idea sounds horrible to me. I like to have that control over my own calendar. What I often do is plan ahead at the end of the session, so I usually have two appointments in the future. I'm guessing if you could put on AI at the moment you're making appointments, and it could put them in the calendar straight away and send a confirmation email, that would save me maybe 30 seconds. So it would be possible, but what I do now already works pretty well: we plan at the end of the session, I put them in the calendar, and clients automatically receive a confirmation email and a reminder the day before. So I don't need to do that myself. I'm open to ideas though, if I could do this part in an easier way, then sure.

I: You mentioned that the idea of clients being able to plan their own appointments would be horrible. That's describing an existing system that your colleagues are already using, right?

P: Yeah, some colleagues are using it, but a lot of them don't because they don't like it. It's always nicer to have control. If this Friday, for example, I only had appointments in the morning and somebody wanted to make another appointment, I would first give them the choice of the first slot in the afternoon instead of the last one, because otherwise I would have a hole in my calendar of two or three hours. So I try to keep that control over my calendar and over the rest of my day.

I: Now for the next question, this is about psychoeducation and between-session tools. What are your thoughts on using AI for psychoeducation or therapeutic exercises between sessions?

P: I think you could do that. There are already a lot of things online you could use, like e-learning that clients can do. I quite often give clients exercises to do in the meantime that I explain to them and they can do themselves. Sometimes clients have a hard time remembering to do something, so what we sometimes do in the session is have them put reminders in their phone so they won't forget the exercise. Maybe AI could help with that, giving them reminders or

something, but I think there are already programs on the market that help with that. So I'm not really sure why I would necessarily need AI for it, but you could use it for that.

I: What type of homework do you give them? Is it an e-learning environment or more physical world homework?

P: It depends on what we're doing in the session. What I quite regularly do is, after one or two sessions, there's usually something happening, maybe anxiety in a certain situation, a fear of failure, or something like that. The question at the beginning is often why the client is experiencing that, so we want to figure out which circumstances in their lives are part of why this is occurring. What I often do is look back into their lives chronologically and see what has happened over the years. Sometimes there are certain years they don't really remember, or I ask them what they were like as a kid, whether they always tried to do really well at school. Some people know that really well, but others aren't really sure. So sometimes an assignment can be to have a conversation with a parent or somebody else who knew them at that time, to get some information on what they were like. Exercises can also be more behavioral therapy type, where people start practicing things they find scary. We often start with really small exercises and slowly make them bigger, exposure exercises. Sometimes you can do that together in a session, but sometimes they just start by themselves really small. I also work with ACT a lot, acceptance and commitment therapy, and for that I have a lot of exercises as well. For example, making a list of all the rules they have for themselves. A lot of people have rules like how they should live, what they should and shouldn't do, that they should always be tough and strong and never cry in public and always be nice. So sometimes I give them an exercise to note down as many of those rules as they can think of. So it varies: sometimes it's really doing things, sometimes writing things down, practicing things, focusing on their feelings, or mindfulness exercises.

I: What would you think about AI being used for psychoeducation or between sessions if it contradicts you as a therapist?

P: I think it's probably very easy to contradict a psychologist because there are so many different ways to do things. If you look at things from the behavioral therapy side or from the acceptance and commitment therapy side, there are some things in both that clash a little bit, but they're both methods that work well. Actually, almost all of the most used methods in psychology have roughly the same success rate, and it's mostly the therapists themselves who make the difference through the bond and relationship they have with the client, which is what determines a successful outcome. So I think it's very easy for AI to contradict me, but I would be curious on which grounds and from which point of view. If it just says this is wrong, you should do that, then you can't really do anything with it. But if there's a good explanation for why you could look at it differently, then sure, I'd be open to that. I'm guessing you do the same thing in intervision with colleagues, where they also see things differently and that's really helpful for gaining perspective. So if AI could give me a different perspective with a solid foundation, that would be good. If it gave me another perspective without any foundation, I would probably just let it go.

I: So moving on to the next question, this is about risks and limitations. You already mentioned a few, like data privacy and transparency, the idea that if you get an answer from AI there has to be some reasoning behind it and it can't just say this is the answer without explanation. Are there any other risks or limitations you think could come up with using AI?

P: I think it's mostly the interpretations of things. I'm not sure if AI has the capacity to take into account all of these different contexts and different types of people, because in psychology you learn a lot about how things work for a lot of people. But when you do individual work, it's always with an individual who sometimes has the same experience as that group, but often it's also slightly different, because all of those things are the products of maybe tens of thousands of people and their collective experience, which is always different from one individual experience. That makes it really hard, because I think AI mostly uses more collective, general things. It's just really hard to use that for one individual and really take their perspective into account. So interpretations and the sensitivity part of sessions, I think that's really hard for AI, because it mostly works with information that you have to put into written texts, and there's quite a lot of information that you cannot really put into written texts well. For example, sometimes in sessions there's something happening between you and a client where the client has patterns and responds in a certain way, but you as a psychologist also have your own patterns and sometimes react to that. Then you get a very interesting dynamic that you can talk about with the client. But I don't think AI has the capacity to understand that, because then you have to understand the whole history of both the psychologist and the client. I think that's too many individual experiences, too hard for AI, at least at the moment.

I: Then for the next question, this is about client perspective and therapeutic alliance. How do you think clients respond to AI being used in their treatment?

P: I'm not really sure, I haven't talked about it much with clients. I would also have to ask the company I work with how their pilot is doing, because they are running a pilot. I'm guessing those psychologists have to ask their clients whether it's okay if AI listens to the session, and I haven't heard any results about the pilot yet. All I can say is what I think people would think, which is pretty subjective. When I've talked to some people about using AI for summaries of sessions, I think I straightaway gave my own perspective, that at the moment I would not do that because of the confidentiality part. Most people agreed with me, but I'm not sure whether they would have agreed as readily if they hadn't heard my opinion first. So I don't really know how clients would respond.

I: If we have a score of 1 to 10 on how likely you think clients would be willing to use AI in their sessions, what would you say?

P: I think that would really depend on how I explain it. If people trust their psychologist and you have a very clear objective for using it and why it's helpful, then I think maybe 60 or 70% would say it's fine. If you can take away their concerns, then it might be fine. So I think you can steer that pretty well as a psychologist.

I: It would definitely be important to explain why you would want to use AI and what benefits it would bring.

P: Yeah.

I: For the last question, this is about professional identity, autonomy and future conditions. Which aspects of your work should remain strictly human, and what would you need to feel confident about using AI safely in practice?

P: I think they're also experimenting with having an AI person do the whole session, and I think that would be hard. It might quickly feel a bit fake. What most clients need is to feel heard by someone who is really there for them and takes the time for them, and I think it would be harder to achieve that with an AI. So in sessions themselves I think it's important to have a person. Doing it online doesn't really change that, because it's still another person sitting in front of you who sometimes says something weird or makes a mistake and you can laugh together about it. I think conversations with a human are not always to the point, sometimes you have a tangent and that's okay, you go back to the conversation and that helps keep it natural. So that part should definitely remain human.

I: And what would you need to feel confident about using AI safely in practice?

P: You would need to understand completely where the data goes, whether it's stored somewhere, whether there's a way to hack it. I think AI is also really intangible for a lot of people, they have no idea what happens to all of this information. Most people know that the information AI has comes from the internet, from different websites, but there's so much rubbish on the internet. So how can you be sure the foundation is good? You would also need to know where it gets its information from. You would need a lot more understanding, and to be sure that the data is completely secure, but that's hard in this world.

I: Do you have anything else to add?

P: No.